Pathology

In tinea versicolor, the organisms inhabit the stratum corneum. There is usually no dermal infiltrate. Yeast may be observed with hematoxylin and eosin staining alone, and PAS staining is confirmatory. Microscopic examination reveals clusters of oval budding yeast cells $3.5 \times 4.5 \mu \mathrm{m}$ in size, along with short, septate, and occasionally branching hyphae. This appearance is classically described as "spaghetti and meatballs" (Fig. 189-11). In *Pityrosporum* folliculitis, organisms are found in widened follicular ostia mixed with keratinous material. Rupture of the follicular wall can lead to a mixed inflammatory response and foreign-body giant cell reaction.

Diagnosis

The clinical appearance of tinea versicolor is typically characteristic, and diagnosis is confirmed by KOH examination. Wood's lamp examination may reveal yellowish fluorescence of the affected skin.

Treatment and Prognosis

Tinea Versicolor

Multiple topical agents are effective for treating tinea versicolor. The most commonly used is 2.5% selenium sulfide shampoo, applied daily for 2 weeks, left on for at least 10 minutes, then rinsed off. Maintenance therapy with monthly application one to two times helps prevent recurrence. Topical azole antifungals are also effective. Ketoconazole 2% shampoo is lathered onto affected areas and left for 5 minutes, repeated for 3 consecutive days. Topical terbinafine 1% solution, applied twice daily for 7 days, has achieved cure rates exceeding 80%.

For extensive disease, frequent recurrences, or failure of topical therapy, systemic treatment is recommended. Oral ketoconazole (200 mg daily for 7 days) or oral itraconazole (200–400 mg daily for 3–7 days) is highly effective. A single 400 mg dose of itraconazole has shown over 75% efficacy and, in one study, was as effective as a 1-week course. Fluconazole is also effective, with a single 400 mg oral dose being sufficient.

Pityrosporum Folliculitis

Pityrosporum folliculitis responds well to systemic antifungal therapy. Although controlled trials are limited, oral ketoconazole (200 mg daily for 4 weeks), fluconazole (150 mg weekly for 2–4 weeks), and itraconazole (200 mg daily for 2 weeks) are effective treatment options.

Key References

- Gupta AK et al: Skin diseases associated with *Malassezia* species. *J Am Acad Dermatol* 51:785, 2004
- Gupta AK, Bluhm R, Summerbell R: Pityriasis versicolor